8.12 Syndrome of Inappropriate ADH (SIADH)*

Assessment	Nursing Diagnosis	Plan / Expected Outcome	Nursing Interventions	Evaluation
Subjective • Family report confusion **Objective** • Na^+ 122 mmol/L • Concentrated urine • Weight gain without edema	**ACTUAL** Excessive fluid volume related to water retention as evidenced by hyponatremia and weight gain.	• Sodium rises gradually to normal.	• Restrict fluids as prescribed. • Weigh daily; keep a strict intake and output chart. • Correct sodium slowly as ordered.	• Considered met as sodium rises.
Subjective • Family say, "He is confused." **Objective** • Disoriented	**ACTUAL** Acute confusion related to hyponatremia as evidenced by disorientation.	• Oriented as sodium corrects.	• Do neurological checks every 4 hours. • Reorient him often. • Report seizures.	• Goal reached once he is oriented.
Subjective *No specific complaint* **Objective** • Severe hyponatremia • Confused	**RISK** Risk for physical injury as evidenced by risk factors: seizures and falls associated with hyponatremia.	• No injury.	• Put seizure precautions in place. • Put fall precautions in place. • Keep the bed low.	• Met if no injury occurs.

* **References for this care plan:** [32], [35], [118], [128]. Full entries are listed in the References section of the CPMN book.